

Policy on Fire and Non-Fire Emergencies

«Hospital Name»

Document control

Document No.	HCO/FMS/POL/07	Version	2.0
Issue No.	01	Review due	One year from implementation
Date created	_____	Date of implementation	_____
Prepared by	Engineering In-Charge Name: _____ Signature: _____		
Reviewed by	Quality Coordinator Name: _____ Signature: _____		
Approved by	Medical Superintendent Name: _____ Signature: _____		

A blank marked _____ must be completed before issue.

Statement of intent

The organisation has plans for fire and non-fire emergencies within the facilities.

1. Purpose

This policy sets out how «Hospital Name» plans for fire and non-fire emergencies. It covers:

- Fire detection, abatement, containment and evacuation
- Non-fire emergency identification and management
- Displayed exit plans
- Regular mock drills
- Maintenance of fire-related equipment and infrastructure

This policy covers fire and non-fire emergencies specifically. Related duties — like patient assessment, clinical care, medication management, patient rights, infection control, quality and safety monitoring, or hospital governance — are covered in the hospital's other policies, not repeated here.

Words marked like this are defaults. A blank marked _____ must be filled before issue.

2. Scope

This policy applies to «Hospital Name» staff involved in facilities, utilities, medical gases, and fire or non-fire emergencies, including:

- Engineering, biomedical, nursing and departmental leaders

- The Engineering In-Charge
- The Medical Superintendent
- Departmental leaders
- The Quality Coordinator

This policy covers fire and non-fire emergencies specifically. Related duties are covered in the hospital's other policies, not repeated here, including:

- Patient assessment
- Clinical care
- Medication management
- Patient rights
- Infection control
- Quality and safety monitoring
- Hospital governance

3. Policy standards

«Hospital Name» has implemented fire plans covering detection, abatement, containment and evacuation, with qualified personnel and current NABH fire-safety measures. Non-fire emergencies are identified and planned for, with NDMA/State/District guidelines as a reference. Exit plans are documented and displayed on every floor. Mock drills are held at least twice a year. Fire-related equipment and infrastructure are maintained under an active plan.

Staff follow written guidance and keep the records listed in the traceability table.

4. Non-negotiable rules

1. Do not occupy a patient-care floor without an implemented fire plan covering detection, abatement, containment and evacuation — with qualified personnel, current NABH fire-safety measures, smoke-control provisions and emergency illumination in place.
2. Do not occupy a patient-care floor without a documented and displayed exit plan on that floor, including near lifts and inside enclosed rooms and laboratories; exit doors must remain open or have push bars.
3. Do not operate without written plans for the non-fire emergencies this hospital has identified (at minimum earthquake, flood, structural collapse, utility failure and toxic leak), developed with reference to NDMA/State/District guidelines.
4. Do not let six months pass without at least one mock drill testing the full fire or non-fire emergency plan; each drill uses simulated, not real, patients and is followed by a debrief and corrective action.
5. Do not operate fire-related equipment and infrastructure without an implemented maintenance plan covering inspection, testing, preventive and breakdown maintenance.
6. Do not bypass the stop-work authority in section 6 when the trigger conditions are met.
7. Staff who see a FMS.7 rule broken report it the same shift to the Engineering In-Charge or the Medical Superintendent.

5. What we do

5.1 The organisation has plans and provisions for early detection, abatement, containment of fire and evacuation in the event of fire emergencies.

«Hospital Name» has plans and provisions for early detection, abatement, containment of fire and evacuation: a fire plan (inflammable items, explosion, short-circuit, negligence or incompetence), qualified personnel, current NABH minimum fire-safety measures, smoke control, training, mock-drill schedules including table-top, drill records, displayed exits, evacuation of patients, staff and visitors, and emergency illumination. The hospital establishes liaison with civil and police authorities and the fire brigade as required by law.

Occupying a patient-care floor without required detection, abatement or evacuation provision is a stop-work trigger (section 6).

5.2 The organisation has plans and provisions for identification, and management of non-fire emergencies.

The organisation has plans and provisions for identification and management of non-fire emergencies (examples the Guidebook names include earthquake, flood, toxic leak, structural collapse, utility failure, boiler burst, violence, stray animals). National Disaster Management Authority / State / District guidelines are referred to. Liaison with civil, police and fire authorities is as required by law.

Portal wording “identification, and management” is kept as printed.

5.3 The organisation has a documented and displayed exit plan in case of fire and non-fire emergencies.

A documented exit plan is displayed on each floor, especially near lifts and inside enclosed rooms and laboratories. Exit doors remain open or have push bars. Fire signage follows the fire service and/or National Building Code. Refuge areas are signed and maintained where they apply.

A patient-care floor with no displayed exit plan is a stop-work trigger (section 6).

5.4 Mock drills are held at least twice a year

Mock drills are held at least twice a year (minimum; more often if this hospital defines). This covers fire and the important non-fire events this hospital names. A table-top exercise or a mock drill may be used; at a minimum one mock drill every six months tests the whole plan, not only awareness. Simulated patients, not real patients, are used. Variations are debriefed and corrected.

A year with fewer than two drills is a gap under this element.

5.5 There is a maintenance plan for fire-related equipment and infrastructure.

There is a maintenance plan for fire-related equipment and infrastructure: inspection, testing, preventive and breakdown maintenance, following the manufacturer and applicable statutory recommendations.

The Engineering In-Charge holds the plan and last service evidence.

6. Stop-work authority

Do not occupy a patient-care floor when required fire detection, abatement or evacuation provision is not in place, or when the exit plan for that floor is not displayed.

Stop-work applies to placing or keeping patients on that floor as a planned location of care. Immediate life-saving evacuation and life-saving care continue.

The person who stops tells the Engineering In-Charge and the Medical Superintendent the same shift. Refusing to occupy an unprotected floor is not a disciplinary matter.

7. Governance and responsibility

Role	Responsibility
Medical Superintendent	Accountable that FMS.7 is resourced and followed.
Engineering In-Charge	Owns day-to-day implementation and records for this standard.
Quality Coordinator	Audits this policy quarterly; holds training acknowledgements.
departmental leaders	Run the department-level duties this standard names.

8. Quality monitoring (RCA → CAPA)

The Quality Coordinator audits this policy quarterly. The audit reviews:

- Records for a sample of this standard's objective elements, checked against the What-we-do steps.
- Documentary evidence is on file for each asterisked objective element in the sample.
- CORE objective elements show no critical gaps in the sample.
- Stop-work events (if any) are logged with outcome.

Root-cause analysis is required when a gap found in this audit remains open beyond 90 days.

This policy is reviewed annually, and sooner after a related facility change, utility failure, equipment recall or fire-plan change.

9. Training and staff acknowledgement

Staff covered by this policy are trained at induction and once a year after that. Training covers the What-we-do steps, non-negotiables and stop-work (if present).

Staff acknowledgement

I have read the Policy on Fire and Non-Fire Emergencies of «Hospital Name». I will follow the processes described.

Name: _____ Designation: _____

Department / floor: _____ Date: _____

Signature: _____

(One row per staff member. The Quality Coordinator holds signed acknowledgements with the induction record.)

10. Distribution

This policy is distributed to:

- Medical Superintendent
- Engineering In-Charge
- Quality Coordinator
- departmental leaders
- staff covered by FMS.7

11. Abbreviations

Abbreviation	Meaning
CAPA	Corrective and Preventive Action
CORE	Core objective element (NABH)
FMS	Facility Management and Safety (NABH Hospitals 6th Edition chapter)
HCO	Hospital (Full Accreditation programme under NABH Hospitals 6th Edition)
NABH	National Accreditation Board for Hospitals and Healthcare Providers
OE	Objective Element

12. Traceability to NABH HCO Full Accreditation 6th Edition FMS.7

This table is an index. It is not how the policy is organised. An asterisk in the Level column means documentation of the process is required.

OE	Level	Requirement	Where	Responsible
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			addressed	
FMS.7.a	CORE*	The organisation has plans and provisions for early detection, abatement, containment of fire and evacuation in the event of fire emergencies.	Section 3; 5.1; Section 6 Stop-work	Engineering In-Charge
FMS.7.b	CORE*	The organisation has plans and provisions for identification, and management of non-fire emergencies.	Section 3; 5.2	Engineering In-Charge
FMS.7.c	Commitment	The organisation has a documented and displayed exit plan in case of fire and non-fire emergencies.	Section 3; 5.3; Section 6 Stop-work	Engineering In-Charge
FMS.7.d	Commitment	Mock drills are held at least twice a year.	Section 3; 5.4	Engineering In-Charge
FMS.7.e	Commitment*	There is a maintenance plan for fire-related equipment and infrastructure.	Section 3; 5.5	Engineering In-Charge

13. Required Records / Evidence Checklist

Records the hospital holds under this policy, listed by objective element.

FMS.7.a — The organisation has plans and provisions for early detection, abatement, containment of fire and evacuation in the event of fire emergencies.

- Written fire-safety plan covering detection, abatement, containment and evacuation, naming qualified personnel and current NABH minimum fire-safety measures.

- Mock-drill schedule and drill record, including table-top exercises.
- Displayed evacuation plan and emergency-illumination check record.

FMS.7.b — The organisation has plans and provisions for identification, and management of non-fire emergencies.

- Written non-fire-emergency plan (for example earthquake, flood, toxic leak, structural collapse, utility failure, boiler burst, violence, stray animals).
- NDMA, State or District guideline reference used.
- Liaison record with civil, police and fire authorities.

FMS.7.c — The organisation has a documented and displayed exit plan in case of fire and non-fire emergencies.

- Exit plan displayed on each floor, near lifts and inside enclosed rooms and laboratories.
- Exit-door check record (open or push-bar) and fire-signage reference (fire service or National Building Code).
- Refuge-area signage and maintenance record, where applicable.

FMS.7.d — Mock drills are held at least twice a year.

- Mock-drill record showing at least two drills a year covering fire and the named non-fire events.
- Debrief and corrective-action record for variations found.
- Confirmation that simulated, not real, patients were used.

FMS.7.e — There is a maintenance plan for fire-related equipment and infrastructure.

- Written maintenance plan for fire-related equipment and infrastructure.
- Inspection, testing, preventive and breakdown maintenance log following manufacturer and statutory recommendations.
- Last-service evidence on file.

14. References

- National Accreditation Board for Hospitals and Healthcare Providers (NABH), Accreditation Standards for Hospitals, 6th Edition (January 2025) — Facility Management and Safety, standard FMS.7.
- NABH Guidebook to Accreditation Standards for Hospitals, 6th Edition — FMS.7 interpretations.
- National Accreditation Board for Hospitals and Healthcare Providers (NABH). Advisory for Minimum Essential Fire Safety Measures, 20 September 2022. Referenced as the current NABH fire-safety measures standard.
- National Disaster Management Authority (NDMA), Government of India. National Disaster Management Guidelines — Hospital Safety. Referenced for identification and management of non-fire emergencies, alongside applicable State and District Disaster Management Authority guidelines.

- Bureau of Indian Standards. National Building Code of India (current version). Referenced for fire signage and exit-plan requirements, alongside applicable fire service norms.

Disclaimer

This document is a template prepared for the guidance of «Hospital Name» and must be reviewed, adapted and formally approved by «Hospital Name» before use. Every entry marked _____ must be completed before the document is issued.

The requirements in this document are accreditation requirements of the NABH Accreditation Standards for Hospitals, 6th Edition, not duties under a named Act of Parliament. This policy does not import the Consumer Protection Act, 2019, the Clinical Establishments Act, 2010, or the Mental Healthcare Act, 2017 as a checklist. Statutory duties that arise under other documents of «Hospital Name» remain those documents. «Hospital Name» is responsible for verifying any statutory duty that applies to it; this document does not constitute legal advice.

«Hospital Name» remains responsible for verifying that it is current and consistent with the edition of the accreditation standard against which it is being assessed. The clinical and technical content reflects recognised national and international guidance current at the date of preparation.

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